

Colonoscopy Procedure Report

GI Associates of Boston | Dr. Elena Hartley, MD

Patient: Jane Doe | DOB: 1988-03-14 | MRN: 00142837

Procedure date: 2026-04-22 Endoscopist: Dr. Elena Hartley, MD

INDICATION

Known ulcerative colitis with worsening symptoms over 6 weeks. Bloody diarrhea 5-6x/day. Evaluate disease extent and severity for treatment escalation.

FINDINGS

Extent of disease: Left-sided colitis (E2, Montreal Classification)

Mayo Endoscopic Score: 3 (severe -spontaneous bleeding, deep ulceration)

Rectum: Severe erythema, friability, continuous ulceration with contact bleeding. Pseudopolyps noted. No normal mucosal pattern visible.

Sigmoid colon: Moderate-severe inflammation. Granular mucosa, loss of vascular pattern, multiple shallow ulcerations 3-8mm.

Descending colon: Mild-moderate inflammation at junction. Transition zone at 55cm.

Transverse, ascending colon, cecum, terminal ileum: Normal appearance. No skip lesions.

BIOPSIES

4 biopsies taken from rectum (active inflammation, cryptitis, crypt abscesses).

2 biopsies from sigmoid (active chronic inflammation).

2 biopsies from descending colon junction (transition zone).

Pathology pending -see separate pathology report.

IMPRESSION & PLAN

Active moderate-to-severe left-sided ulcerative colitis (Mayo 3).

Current mesalamine monotherapy inadequate. Consider biologic therapy.

Prednisone bridge 40mg/day with planned taper.

Repeat colonoscopy in 12 months or sooner if symptoms worsen.